One health concern. Several decisions

Where do I start?

A resident must work out the next step before reaching care.

THE PROBLEM

Finding care.
Understanding coverage

Which provider should I see?

The resident needs an appropriate starting point and an available appointment.

What care can I use?

Covered services and sponsored visits have their own eligibility rules.

Problem hypothesis. Resident interviews and workflow tests must establish the actual friction.

THE PROBLEM

A prescription leaves another decision

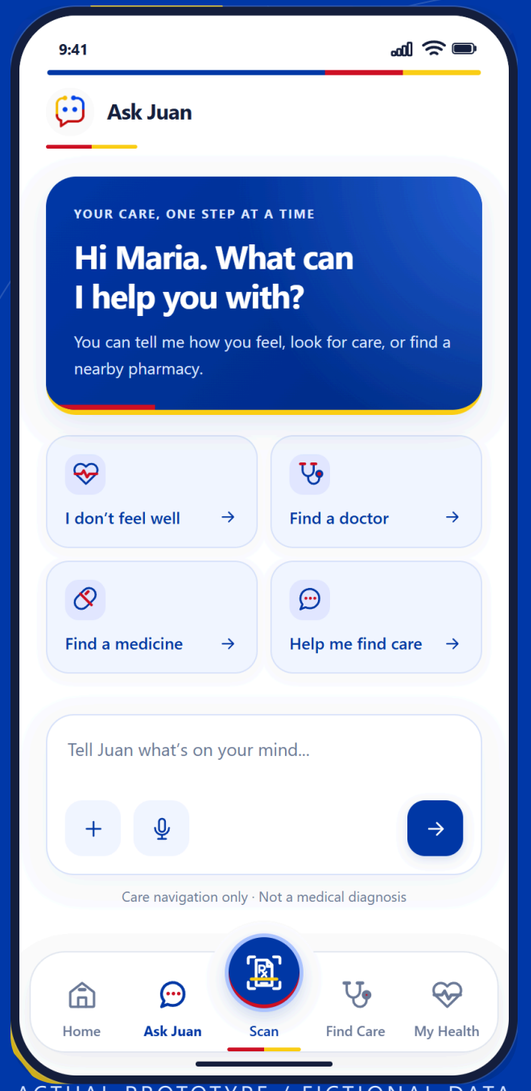
Where can I find my medicine?

A resident still needs to locate a pharmacy and confirm availability before travelling.

Problem hypothesis. Current medicine listings in the prototype are fictional.

One concern. A clearer path to care

Healthcare navigation and institutional care coordination.
Davao City first.



ACTUAL PROTOTYPE / FICTIONAL DATA

Davao City is our starting point

1.85M

Davao City residents

A large local population gives us a focused place to test healthcare navigation.

Exact 2024 census count: 1,848,947.
Population provides context, not an E-JUAN user forecast.

PSA 2024 Census of Population. Population as of 1 July 2024.

E-JUAN

Households carry a substantial share

41.2%

of Philippine current health spending

Households paid this share out of pocket in 2025.

Clear care and funding choices matter.
This is national spending context, not a Davao-specific rate.

PSA 2025 Philippine National Health Account. Share of current health expenditure.

A regional prospect universe

54

provincial, city and municipal LGUs

Potential institutional buyers in Davao Region.

5 provinces + 6 cities + 43 municipalities.
Each buyer still needs a verified need, budget and purchasing authority.

PSA Philippine Standard Geographic Code. Possible prospects. No signed E-JUAN customers.

An institution-funded software license

₱10,000

proposed monthly license per institution

An LGU or healthcare program sponsor would pay for coordination.

Clinical payments remain separate.
E-JUAN assumes a fixed license, with no percentage of professional fees.

Illustrative assumptions from the original concept. Pricing, costs and willingness to pay remain unvalidated.

Care spending and software revenue

Assume 100 consultations at ₱500 each.

₱50,000

Clinical services

Paid separately to providers

+

₱10,000

E-JUAN software

Illustrated revenue before costs

=

₱60,000

Sponsor spending

Before additional charges

Illustrative assumptions from the original concept. Pricing, costs and willingness to pay remain unvalidated.

Software cost per visit

₱100

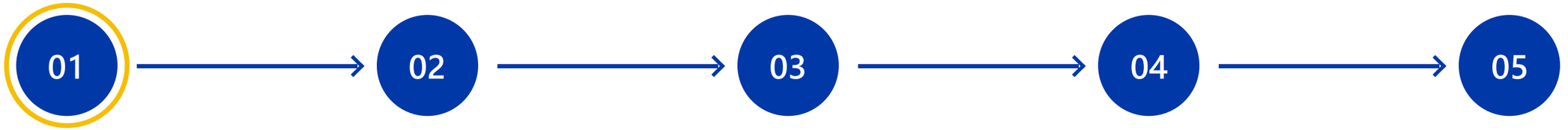
per visit at 100 consultations per month

₱10,000 monthly license ÷ 100 consultations.

A sponsor must see enough administrative value to justify this cost.
Delivery costs and profitability still need validation.

Illustrative assumptions from the original concept. Pricing, costs and willingness to pay remain unvalidated.

Five steps through care



ASK

Describe the concern

MATCH

Compare care options

CHECK

Review funding

CONSULT

Meet a clinician

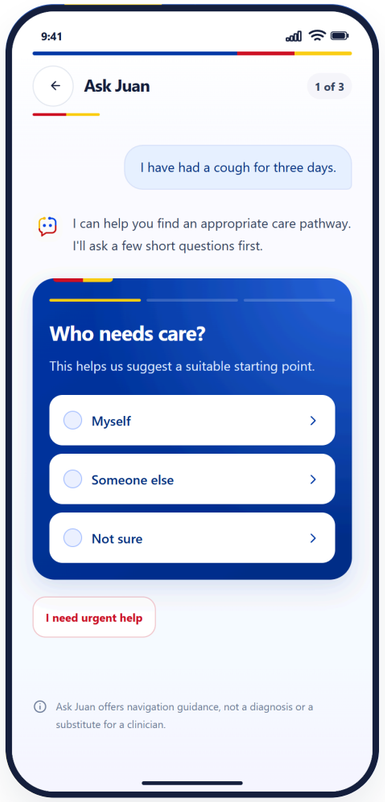
LOCATE

Find medicine

An existing prescription can start directly at medicine search.

E-JUAN

A clearer starting point



Three short questions

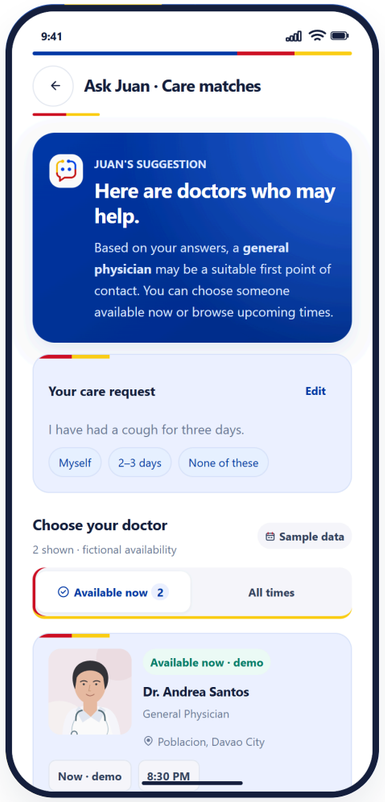
Who needs care, how long it has lasted, and any warning signs.

Navigation without diagnosis

Urgent warning signs lead to separate urgent-care guidance.

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Compare appropriate care options



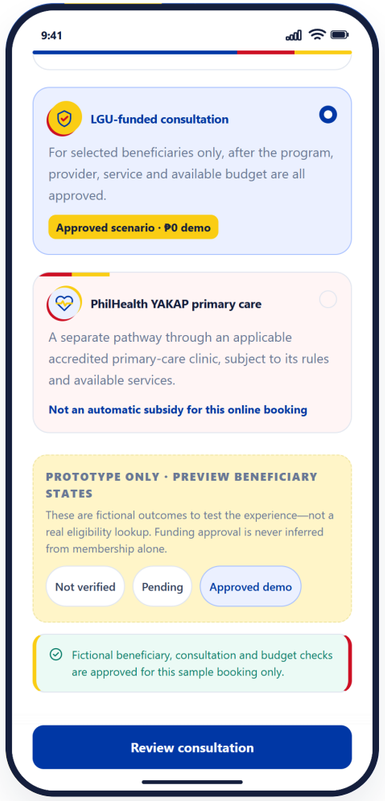
Doctor and appointment choices

The prototype shows fictional providers and sample slots.

Public or covered care first

This intended ranking requires verified participating services.

Review the funding pathway



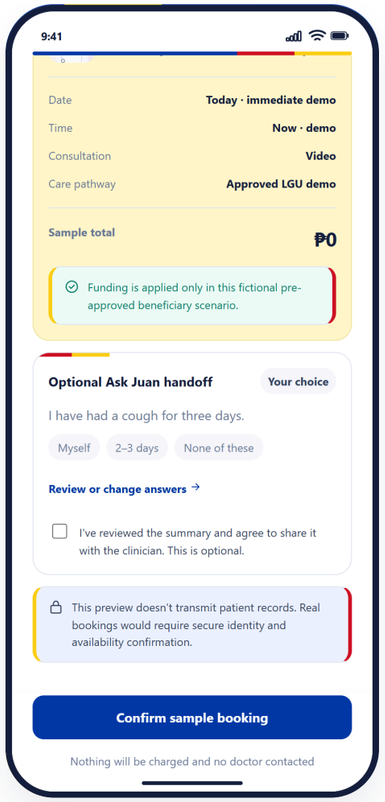
Standard or sponsored care

Sponsored visits require beneficiary, provider, service, allowance and budget checks.

A separate YAKAP pathway

PhilHealth membership alone does not approve an LGU-funded consultation.

Review details and choose consent



Appointment details before confirmation

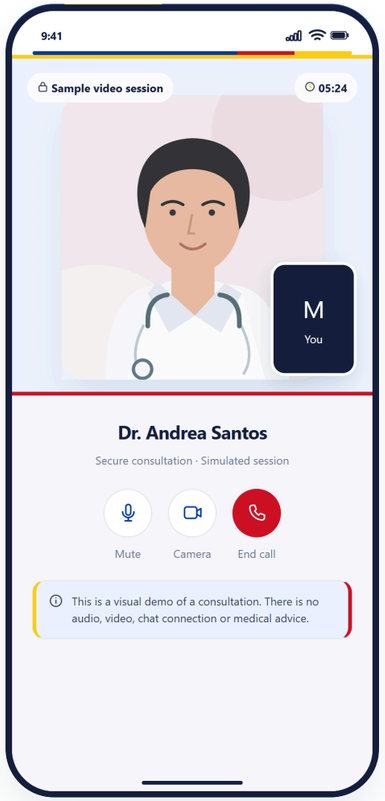
Review provider, time, mode, funding and sample total.

Optional intake sharing

The summary-sharing checkbox starts unchecked.

E-JUAN

The clinician makes the clinical decisions



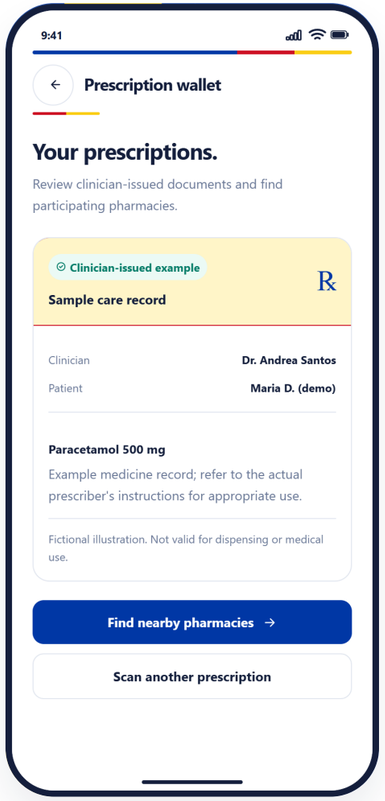
Licensed clinician

Diagnosis and prescribing belong to the authorized care team.

Consultation preview today

The screen simulates a visit using fictional data.

A prescription has a next step



Prescription wallet

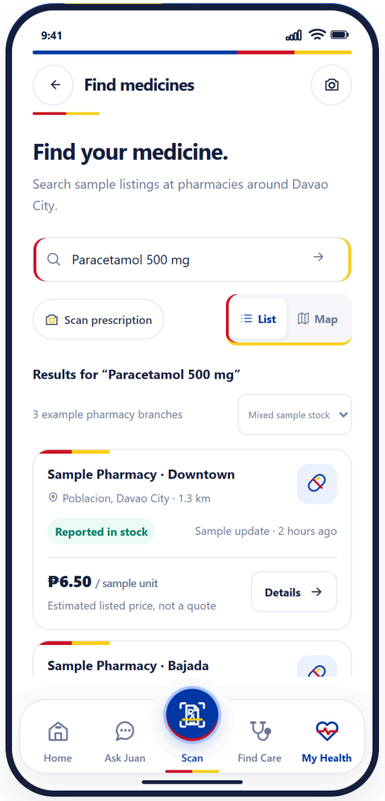
The interface demonstrates how a resident could open a clinician-issued record.

Continue to medicine search

The example links a prescription to nearby sample listings.

Fictional Paracetamol 500 mg record. Invalid for medical use or dispensing.

Confirm medicine before travelling



Compare the listings

Sample prices, distance, stock status and update age.

Confirm availability

A future pharmacy partner must verify stock before the resident travels.

Accountable sponsored visits

Before the visit

Apply beneficiary rules and authorize the service within allowance and budget.

After the visit

Reconcile the completed consultation with its booking and approved payment.

> **Sponsor records cover eligibility, booking and spending.**
> **Clinical access follows the patient’s consent and the care team’s role.**

A specific coordination focus

Existing care services

NowServing and mWell offer digital care.
mWell already has LGU solutions.

YAKAP and Davao City CHO
provide public-care pathways.

E-JUAN's proposed focus

Connect care choices to sponsor rules,
approved visits and spending records.

Add local medicine confirmation
through participating pharmacies.

NowServing · mWell · YAKAP · Davao CHO. Incremental value remains to be tested.

An experience we can test today

23

screens in the local prototype

The interactive demo covers intake, bookings, consent and medicine discovery.

Runs locally without a backend or account.
Connected operations and patient outcomes remain unproven.

Actual repository interface. All records and service interactions are fictional.

What a connected pilot requires

Authorized partners

A healthcare provider, an institutional sponsor and participating pharmacies.

Verified operations

Identity, eligibility, appointments, secure handoff and service reconciliation.

> **Clinical, funding and data-protection arrangements precede live operations.**

A local test with clear measures

Synthetic usability testing, then an authorized Davao City pilot.

Resident journey

Navigation success
Reliable medicine confirmation

Sponsor operations

Staff minutes per booking
Completed visits reconciled

> **The commercial gate: a qualified sponsor willing to pay and renew.**

Expand only after measurable benefit and a feasible institutional purchase. No measured pilot results yet.

A clearer path to care

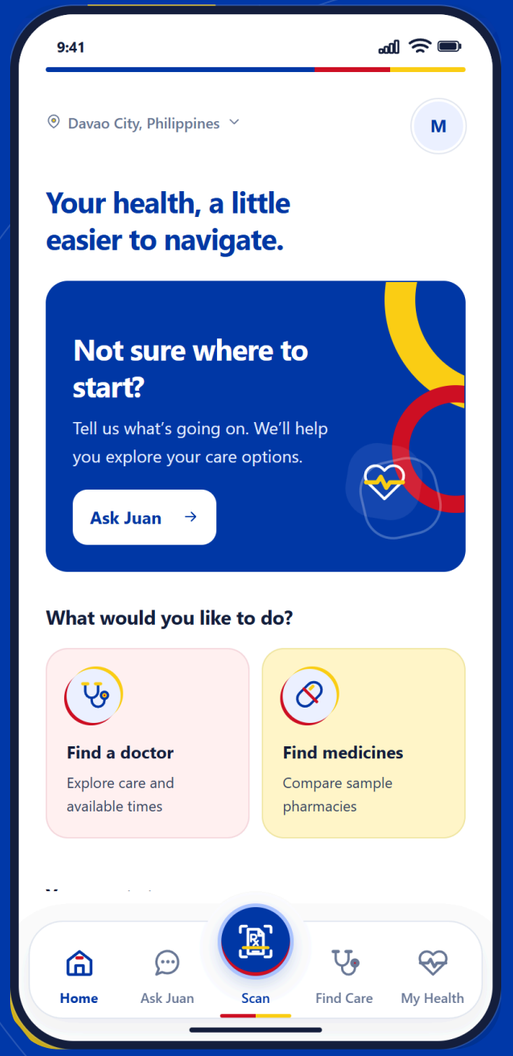
One willing healthcare provider.

One authorized institutional sponsor.

Pharmacy partners.

To validate one credible local pilot.

ALAGANG ABOT-KAMAY



Concept stage. No signed partners, live clinical integrations or proven institutional demand.